

Preeclampsia prevention starts with a shared plan

Identify risk early. Discuss prevention. Keep the plan visible.

01

Assess risk

02

Make a plan

03

Follow through

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Know your risk, your plan, and when to call

FOR PATIENTS

Understand your choices

Learn what preeclampsia means, who may benefit from aspirin, and which symptoms need urgent care.

[Start patient education →](#)

FOR DOCTORS + APPS

Make prevention reliable

Apply risk criteria, counsel patients, and document the decision using a signed paper checklist.

[Go to provider toolkit →](#)

A prevention plan belongs in the conversation and in the chart.

Preeclampsia affects blood pressure and organs

Preeclampsia is a pregnancy condition involving high blood pressure and signs that organs may be affected.

Blood pressure

It may rise even when you feel well.

Organs

The kidneys, liver, brain, or lungs may be affected.

Pregnancy

It can affect the placenta, the organ that supports the baby's growth.

It usually starts after 20 weeks. It can also develop after birth.

Share your history before or early in pregnancy

BEFORE PREGNANCY

Review your health

Discuss prior pregnancy problems and long-term conditions.

FIRST PRENATAL VISIT

Ask about your risk

Bring a medication list and tell your team about allergies.

EARLY PREGNANCY

Write down the plan

Ask whether aspirin is appropriate and when to start.

Prevention starts with a clinical conversation. Do not start aspirin on your own.

Risk screening asks about more than today's symptoms

Medical and pregnancy history

Prior preeclampsia, high blood pressure before pregnancy, diabetes, kidney disease, autoimmune illness, or twins may matter.

Other parts of your story

Your first pregnancy, age, family history, fertility treatment, and access to care also help your clinician assess risk.

On the chart form, choose “not sure” when you do not know. Your clinician will review the answers.

Low-dose aspirin can lower risk for eligible patients

PREVENTIVE TREATMENT

Lower the chance

A small daily dose can reduce the chance of preeclampsia in people at increased risk.

CONTINUED CARE

Keep your checks

Aspirin does not eliminate risk or treat preeclampsia that has already developed.

“Prophylaxis” means treatment used to help prevent a condition.

Agree on the dose and start date with your clinician

81 mg daily is the US guideline dose
for eligible patients

12 WEEKS

Start window opens

BEFORE 16 WEEKS

Preferred timing

28 WEEKS

End of recommended
start window

Guidance continues daily aspirin until delivery. Follow your clinician's individualized instructions.

Tell your clinician about allergies, bleeding, and medicines

Past reactions

Report reactions to aspirin or anti-inflammatory medicines, including breathing problems.

Bleeding or ulcers

Tell your clinician about bleeding, stomach ulcers, or serious liver problems.

Other medicines

Bring a complete list, including blood thinners, over-the-counter products, and supplements.

Your clinician weighs the benefits and risks before recommending aspirin.

Discuss anything that makes the daily plan difficult

01

Access

Can you obtain the medicine?
Tell the team about cost or
pharmacy barriers.

02

Routine

Ask about a reminder that fits
your day and what to do if you
miss a dose.

03

Questions

Share side effects or concerns.
Do not change the dose on
your own.

Try teach-back: “Here is how I will follow the plan.”

Aspirin does not replace prenatal or postpartum checks

Prevention

Follow the aspirin plan if your clinician recommends it.

Detection

Keep appointments and blood pressure checks.
Follow any home-monitoring instructions.

Before leaving, know your follow-up date and who to contact with concerns.

Warning signs need medical care right away

Headache or vision changes

A severe headache that persists or worsens;
blurred vision or seeing spots.

Severe pain or breathing trouble

Severe belly pain, trouble breathing, or chest pain.

**Contact your care team immediately. If you cannot reach them, go to the emergency department.
Call 911 for a life-threatening emergency.**

These are examples, not a complete symptom list. Tell the team you are pregnant or recently gave birth.

Ask your MFM specialist for a written plan

About prevention

- What raises my risk?
- Should I take aspirin?
- What dose, start date, and stop plan should I follow?

About follow-up

- What if I miss a dose or have side effects?
- Do I need home blood pressure checks?
- Who do I call, including after hours?

Bring these questions to your next visit. Ask for an interpreter when needed.

Make the same assessment reliable at every visit

Nadia Kendrick's proposed 12-week initiative standardizes screening, counseling, and aspirin decisions during routine prenatal care.

Project aim

Improve delivery and documentation of evidence-based prevention.

Paper adaptation

A signed chart checklist replaces the proposed EHR smart-phrases workflow.

Source: Kendrick project materials, pp. 1-5 [8]. No completed outcomes were supplied. Paper workflow is the user's requested adaptation.

One high-risk factor supports an aspirin recommendation

Prior preeclampsia

Especially with an adverse outcome.

Multifetal gestation

Twins or higher-order multiples.

Chronic hypertension

Hypertension before pregnancy.

Pregestational diabetes

Type 1 or type 2.

Kidney disease

Autoimmune disease

Such as SLE or antiphospholipid syndrome.

Confirm the history and review contraindications before prescribing.

Assess moderate risk and social context

Multiple factors

First pregnancy; prepregnancy BMI >30; age ≥35; mother/sister with preeclampsia; IVF; personal history factors.

Personal history includes low birth weight/SGA, prior adverse pregnancy outcome, or >10-year pregnancy interval.

Equitable assessment

Black racial identity and lower income are also moderate-risk factors. ACOG/SMFM allows consideration with either alone.

The risk reflects structural inequities and racism, not biological race.

Recommend aspirin with more than one moderate-risk factor; individualize and document the assessment.

Use one form with clear roles and a signed clinical plan

PAGE 1

Patient + front office

Identify the encounter, report history, and route urgent concerns to clinical staff.

PAGE 2

Doctor or APP

Confirm risk, review safety, counsel, and record the aspirin decision.

PAGE 3

Care team + filing

Reconcile follow-up and confirm all pages are filed in the correct chart.

[Download the 3-page chart checklist →](#)

Screen, assess safety, counsel, and record the decision

01

Confirm risk

Resolve “not sure” responses. Document gestational age.

02

Assess eligibility

Review dose, timing, contraindications, and medicines.

03

Counsel

Benefits, limits, risks, access, and patient preference.

04

Sign the plan

Record decision, follow-up, and clinician signature.

A checked box supports the record; it does not replace clinical judgment.

Document why aspirin is or is not used

Project decision categories

Initiated · Continued
Not indicated · Patient declined

Clinical exceptions

Contraindicated · Temporarily held
Pending clarification · Planned start

Record the rationale, actual or planned start date, dose, and follow-up. A justified nonprescription is not automatically a protocol failure.

Measure care processes before claiming better outcomes

01

Screening

Was the risk assessment completed?

02

Counseling

Was appropriate counseling documented?

03

Decision

Was the aspirin decision recorded?

04

Gestational age

When did screening and the decision occur?

Completed checklist ≠ medication adherence ≠ demonstrated reduction in preeclampsia.

Source: proposed 12-week evaluation, project pp. 1-2, 5 [8]. No project results are available in the supplied document.

Keep the clinical chart and research extract separate

Clinical record

Keep the completed form in the correct patient chart under the practice's access and retention policies.

Research extract

Use approved study IDs and de-identified fields. The researcher should not receive names, MRNs, or signed chart pages.

[Download the de-identified audit worksheet →](#)

Practice the next step, not just the facts

Patient scenario

You take prescribed aspirin but develop a severe persistent headache and blurred vision.

What should you do?

Seek medical care immediately. Aspirin does not rule out preeclampsia.

Provider scenario

At 13 weeks, a patient has chronic hypertension. No aspirin contraindication is found.

What should you do?

Recommend guideline-directed low-dose aspirin, discuss benefits and risks, and document the shared plan.

Identify risk early and close the loop

1. **Screen early:** use a complete history and revisit uncertainties.
2. **Confirm eligibility:** consider both risk factors and contraindications.
3. **Optimize timing:** plan the start, dose, and duration explicitly.
4. **Support daily use:** ask about access, concerns, and follow-through.
5. **Keep monitoring:** explain urgent signs and the follow-up plan.

Aspirin is supported; higher doses and implementation need scrutiny

Established

Randomized-trial synthesis supports preventive aspirin in increased-risk pregnancies. US guidance recommends 81 mg.

Dose uncertainty

A 2026 trial did not establish superior efficacy of 162 mg over 81 mg and reported an abortion imbalance.

Next research

Does the paper workflow improve screening, counseling, timely initiation, and equitable access?

Research on higher doses does not make 162 mg the default for this project.

Clinical recommendations have traceable sources

1. [ACOG and SMFM. Low-Dose Aspirin Use for the Prevention of Preeclampsia and Related Morbidity and Mortality. Practice Advisory. December 2021; reaffirmed October 2022.](#)
2. [ACOG Committee Opinion No. 743. Low-Dose Aspirin Use During Pregnancy. Obstet Gynecol. 2018;132:e44-e52. doi:10.1097/AOG.0000000000002708.](#)
3. [Davidson KW, et al.; USPSTF. Aspirin Use to Prevent Preeclampsia and Related Morbidity and Mortality. JAMA. 2021;326:1186-1191. doi:10.1001/jama.2021.14781.](#)
4. [Henderson JT, Vesco KK, Senger CA, Thomas RG, Redmond N. Updated Evidence Report and Systematic Review for the USPSTF. JAMA. 2021;326:1192-1206. doi:10.1001/jama.2021.8551.](#)

The project source describes methods, not results

5. [ACOG. Preeclampsia and High Blood Pressure During Pregnancy. Patient FAQ.](#)
6. [USPSTF. Hypertensive Disorders of Pregnancy: Screening.](#) Final recommendation. September 19, 2023.
7. [CDC. Urgent Maternal Warning Signs and Symptoms.](#) Hear Her campaign. May 15, 2024. Also: [patient conversation guidance.](#)
8. Kendrick N. *Strengthening Preeclampsia Prevention in At-Risk Pregnant Women Through a Standardized Low-Dose Aspirin Prophylaxis Protocol*. Unpublished project materials supplied by user, undated, pp. 1-5. Georgia Southern University. No completed results supplied.
9. [Khander A, et al. Comparison of 162 mg and 81 mg Aspirin for Prevention of Preeclampsia: A Randomized Controlled Trial.](#) *Obstet Gynecol.* 2026;147:87-96. doi:10.1097/AOG.0000000000006100.

Use education to support an individualized care plan

Ask about risk. Write down the plan. Keep follow-up visible.

For your visit

[Print the chart checklist →](#)

Complete it with the care team and retain it in the clinical chart.

For the research team

[Print the audit worksheet →](#)

Use only the approved de-identified extraction process.

Educational material, not diagnosis, a prescription, research consent, or enrollment. Care decisions belong to the treating clinician. This presentation does not collect personal health information.